

CONSULTING PROPOSAL

Health Tech Ai

B2C Awareness, Adoption & Growth Strategy

From zero visibility to sustainable growth - a complete business system



AGENDA

- 1. Executive Summary**
- 2. Market Context & Customer Insights**
- 3. Problem Definition**
- 4. Core Value Proposition (What Health-Tech ai Solves)**
- 5. Target Segment & Wedge Strategy**
- 6. Messaging architecture.
Product Experience & Adoption Model**
- 7. Go-To-Market Strategy (Dual Engine)**
- 8. Growth Plan & Execution Roadmap**
- 9. Unit Economics & Risk Considerations**
- 10. 90 Day Roadmap & Next Steps**



Executive Summary

Situation · Complication · Resolution



Executive Summary - A Business System, Not a Campaign

The product works. The buyer exists. The system to scale it does not.

SITUATION

The market is ready. The buyer is real. Health tech ai is invisible.

- + **61M - 82M**
Americans aged 65+ (by 2050)
- + **75%**
Want to age at home
- + **59M**
Family caregivers in the U.S.
- + **Primary Buyer**
Adult daughters, 40–60
High-income, nearby
- + **100%**
Found AIP Score™ useful
(Mean: 4.04/5, n=27)

COMPLICATION

Discovery solves only 20% of the problem - adoption is the real challenge

- + **Behavioral Barriers**
52% Cost
44% Data privacy
33% AI distrust
- + **Reality Check (What We Got Wrong)**
\$35 CAC - unrealistic
8-min onboarding - Optimistic
“All caregivers” - not a strategy
- + **Real Benchmarks**
\$65–120 CAC
Early-stage health tech
18–25% - Onboarding completion

Without behavioral precision and a clear wedge, the funnel leaks at every stage.

RESOLUTION

A complete business system, designed to drive adoption, not just awareness

- + **Focused Market Entry**
Wedge strategy: early cognitive decline families
Start narrow → build proof → expand.
- + **Behavior-Driven Adoption**
Messaging: guilt → identity → reassurance
Onboarding fixes to improve completion
Designed for real caregiver behavior
- + **Retention & Growth Engine**
3 habit-formation loops
Freemium conversion model
Clinical + digital GTM
- + **Execution Discipline**
CAC sensitivity (3 scenarios)
Honest defensibility assessment
Risk register with clear pivot triggers

Background & Market Context

Market landscape · Platform overview · What 30 NJ caregivers told us



Background -The Market & What Our Survey Found

\$45.8B AgeTech · 59M caregivers · Survey n=30 validated every key assumption

61.2M

Americans 65+
18% of US population

82M

Projected 65+
by 2050 (+42%)

75%

Adults 50+ want
to age in place

59M

Unpaid family
caregivers in US

\$45.8B

AgeTech market
9.2% CAGR

Our Survey Cohort (n=30)

78% Female
38% Age 45–54
69% Caring for a parent
48% HHI \$150K+
56% Within 30 minutes

Primary Buyer

The Adult Daughter Caregiver
Highly involved
Financially capable
Proximity enables action

The Silver Wave

- + Aging population is accelerating 18% of the U.S. is now 65+
- + Care demand is outpacing supply +46% home health demand by 2030 1M+ workers needed - not available
- + Who fills the gap? Families - with no system

AI Adoption Momentum

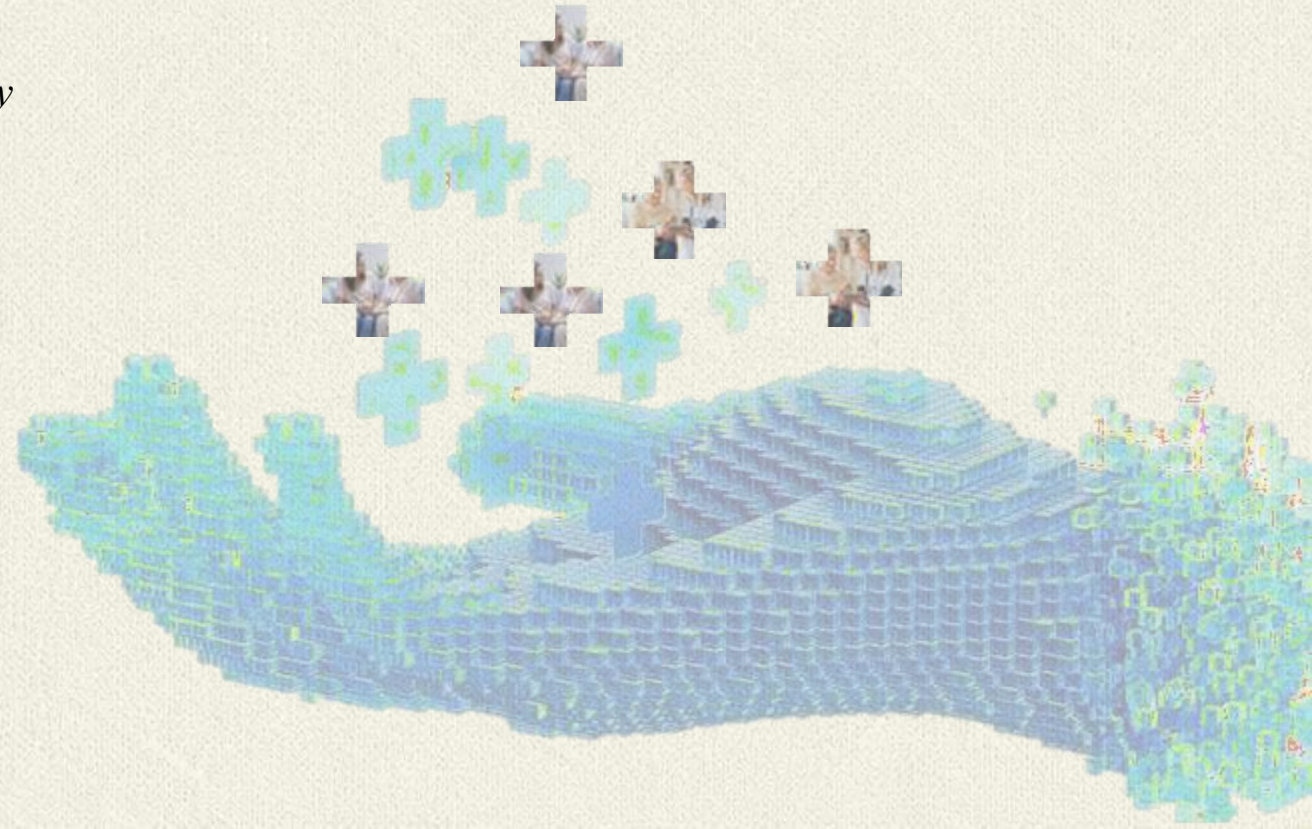
- + Adoption is rising Fast 18% → 30%
Adults 50+ using AI health tools (AARP, 2026)
- + Digital behavior is already established
55% Caregivers use coordination tools
- + Search is evolving “I rely on ChatGPT, Claude, Copilot for medical guidance” (Q11)

The White Space

- + **100%** found the AIP Score™ useful (4.04/5).
- + No one owns this category yet.
- + The opportunity is real - but it must be won through a focused wedge, not broad awareness.

The Problem

Three structural gaps — all three must be solved simultaneously



The Problem - Three Structural Gaps Blocking Growth

Solving one without the others creates a leaky funnel that acquires and churns

01

No Awareness at the Trigger Moment

Insight

40% Cognitive / memory decline
27% Hospital visit
27% Family conversation
27% Proactive concern.

Barrier

Caregiving begins at trigger moments not awareness campaigns. Health tech ai is missing.

Strategy

Focus: families already experiencing cognitive decline

02

Discovery to Adoption Gap (Behavioral Friction)

Insight

52% Cost
44% Data privacy
33% AI distrust
30% Parent resistance

Barrier

These are behavioral barriers — not marketing problems.

Strategy

Onboarding Reality
Only 18–25% complete the AIP Score™
Biggest drop-off: First health data question

03

Clinical is high-trust but slow to activate

Insight

59% Go to doctor / social worker first.
6–18 months Hospital adoption cycle
4–12 weeks HIPAA / BAA negotiation

Barrier

Clinical is not a marketing channel - it's a healthcare GTM challenge

Strategy

Physician contacts
No paperwork
QR code at discharge

Track: QR scans , Score completions
Conversion vs digital

What Health tech Ai Actually Solves

Detect early cognitive decline before it becomes a crisis

Most caregiving tools react **after something happens** Health tech ai is designed for what happens **before**

The core job-to-be-done

- + Families don't know if something is "off" or serious
- + Cognitive decline is **gradual and invisible**
- + Most decisions happen **too late**

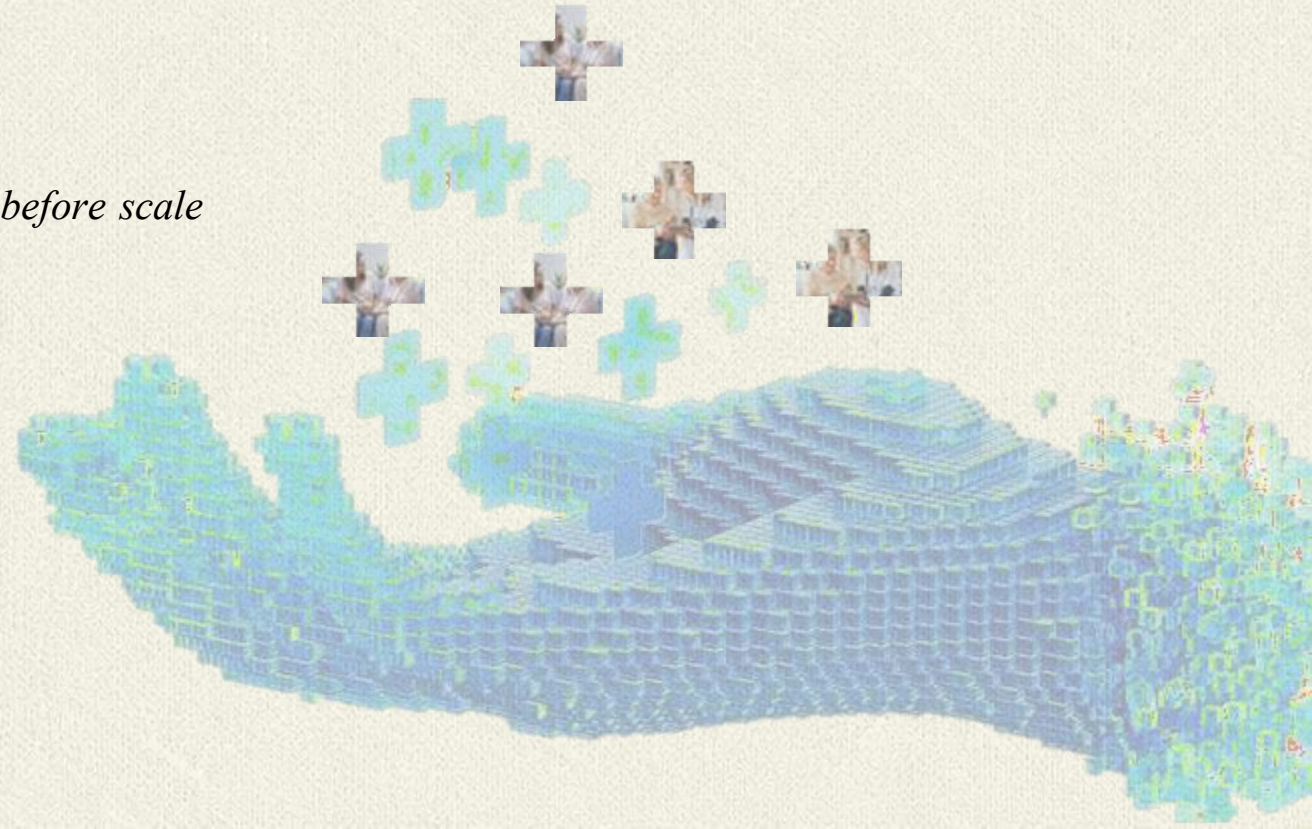
What Health tech ai does differently

- + Tracks subtle behavioral and health signals over time
- + Converts them into a **clear, simple AIP Score™**
- + Identifies **early shifts before they become visible problems**

It is an **early detection and decision system for families**

Target Segment & Wedge Strategy

Where we win first — beachhead market and niche dominance before scale



Target Segment — The First Customer We Win

Early cognitive decline families in NJ



Primary Buyer

Adult daughter (40–60)

High-income, nearby

Situation

Parent showing memory /
cognitive changes

High uncertainty, high
anxiety

Why This segment?

40% triggered by cognitive
decline

Strong emotional urgency

Highest need for early
detection

Wedge Strategy - Win Here First, Then Expand

Primary Wedge

Early Cognitive Decline Families

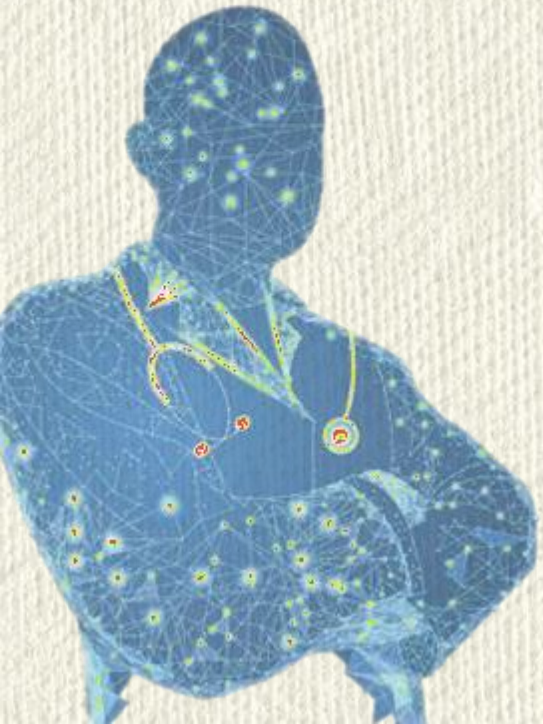
Why we win here?

40% highest trigger
Years (not months) of engagement
Highest emotional urgency

Next expansion

100+ testimonials from
cognitive decline families
High-income caregivers
Post-hospital families

Start narrow → build proof → expand with lower CAC



Messaging Architecture

Guilt · Identity · Denial — the real emotional drivers in caregiving



Messaging Architecture - 3-Layer Emotional Model

Caregiving decisions are driven by guilt, identity, and denial - not logic.

Layer 1

Fear & Guilt

Message:

“Don’t miss the signs”

Use:

Google Ads, social scroll stops,
podcast hooks

Key insight:

Fear drives attention — but must
resolve quickly

Layer 2

Identity & Reassurance

Message:

“You’re the kind of family that
catches things early”

Use:

Landing page hero, score delivery
email, onboarding

Key insight:

People act to protect identity, not just
avoid risk

Layer 3

Trust & Low Commitment

Message:

“A quick look. Nothing to decide
yet. Just the picture’

Use:

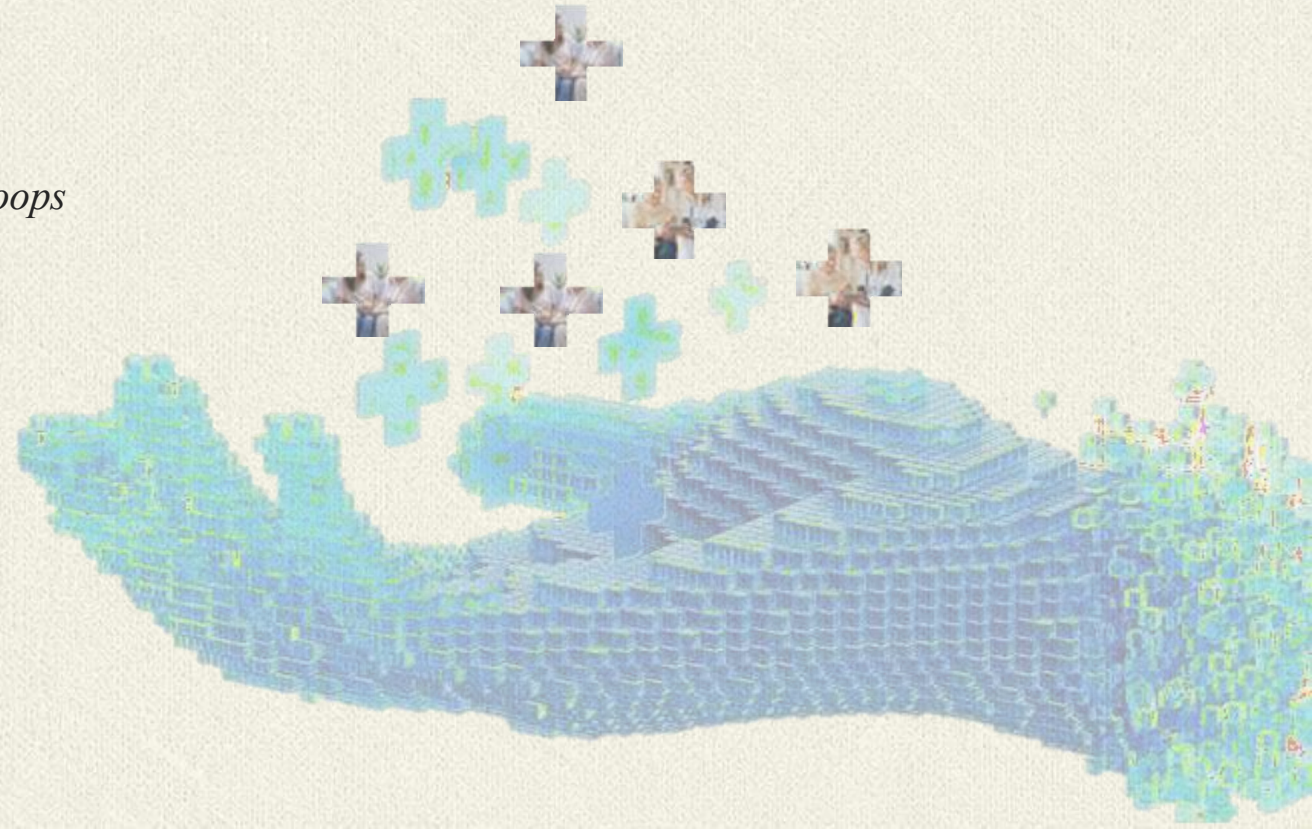
Testimonials, clinical proofs

Key insight:

Reduce pressure to overcome denial

Adoption Strategy & Product Experience

Behavioral friction · Realistic onboarding funnel · Three habit loops



Adoption Strategy - Realistic Funnel & Fixes

Adoption is far lower than assumed and driven by onboarding friction

Only 18–25% of landers complete the full flow The biggest drop-off occurs at the first health data question

Realistic Onboarding Funnel

Land on free score page	100%	n ≈ 1,000	+	<i>From paid search + clinical QR code + referral</i>
Start onboarding (click CTA)	45–55%	n ≈ 500	+	<i>Health app landing page conversion benchmark (Clearbit, 2024)</i>
Pass Q3 (first health data ask)	28–38%	n ≈ 330	+	<i>Drop-off spike at health data input - privacy concern peak</i>
Complete full score flow	18–25%	n ≈ 210	+	<i>Realistic completion from landers (Hotjar health app research, 2024)</i>
Return within 7 days (Loop 1 works)	8–12%	n ≈ 90	+	<i>Cold-start return - powerful weekly pulse</i>
Convert to paid (Day 14)	15% of completers = ~3% of landers	n ≈ 32	+	<i>Distinction matters: 15% of completers convert (~3% of total landers)</i>

*Modeled funnel (based on benchmarks)

4 design fixes to lift completion 40–60%

- (1) Progress bar 'Step 2 of 5' · (2) 'I don't know' on every health Q · (3) Partial score preview after Q3 · (4) SMS re-engagement at 2-hour abandonment

Habit Formation -The 3 Loops That Drive Retention

Email ≠ retention . Habits drive retention..

Loop 1 The Weekly Pulse Weeks 1 - 4

Expected cadence = retention

Cue: Weekly score update

Action: Check score + 1 key change

Reward: Certainty or urgency - reason to return

Missing the update creates a habit of checking

Loop 2 Identity + Reassurance Weeks 4-8

Reinforcement

Cue: 4-week streak

Action: See trend

Reward: “I’m a proactive caregiver”

Identity drives retention

Loop3 Network Months 2+

Churn drops 40–60%

Cue: Share with family

Action: Invite sibling

Reward: Shared certainty

Product becomes harder to leave

Retention is not driven by features — it’s driven by behavior loops.

Clinical Channel Reality

Healthcare GTM is not a marketing problem



Clinical GTM - Reality Check

59% caregivers trust doctors first but clinical activation takes 6–18 months

What Actually Blocks Clinical Adoption

Sales Cycle

6–18 months to close

Legal → IT → department approvals

HIPAA / BAA

4–12 weeks minimum

IT security review

Physician Behavior

Tools must reduce workload - not add to it.

Competitive Advantage

Doctor trust drives **higher conversion quality**

The Honest 90-Day Pilot — Four Steps

Month 1 - Warm Pilot

2–3 doctors

QR at discharge

No paperwork

Month 2 – Measure

QR scans

Score completion

Conversion vs digital

Month 3 – Formalize

If >2x conversion

Start BAA

Define workflow

If Pilot Fails

Clinical = credibility

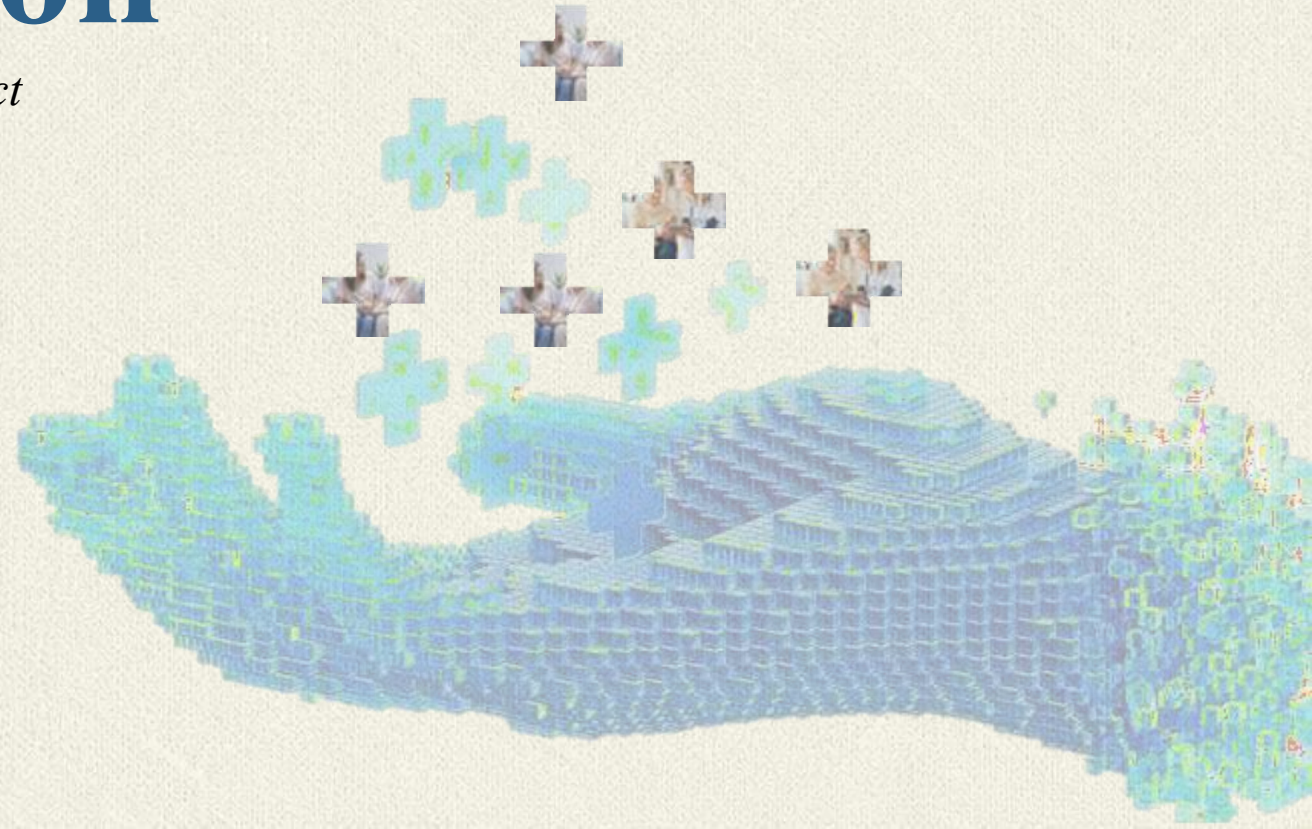
Shift to paid search

Not acquisition

Clinical is a credibility channel first , not a scalable acquisition engine.

Proposed Solution

Five plays · Wedge-focused · Prioritized by 80/20 impact



Growth Plan - 5 Plays, Prioritized

Phase 1 drives ~80% of Year 1 impact

All plays start with the cognitive decline wedge

P1 - Phase 1

Brand Relaunch

Story-first hero (Owner + family)

“Clinically-informed scoring” positioning

Emotion-led messaging sequence

Wedge testimonials front and center

P2 - Phase 2

Freemium Funnel

Free score → habit loops → **\$19.99/mo (Day 14)**

Fix friction (Q3 drop-off + SMS re-engagement)

80% willing to pay < \$50 (*Q20*)
18–25% complete → ~15% convert

P3 - Phase 3

Wedge Search

High-intent wedge keywords only

No broad senior care terms

Google Search + FB caregiver groups

CAC: \$45–65 (base) | \$120–160 (worst)

P4 - Phase 4

Clinical Warm Pilot

M1: Warm contacts, QR at discharge

M2: Measure vs. digital baseline

M3: >2× → **formalize** | else **pivot to credibility**

P5 - Phase 5

Content Engine

4 SEO posts/month (wedge keywords)

YouTube Q&A (2×/month)

Weekly newsletter

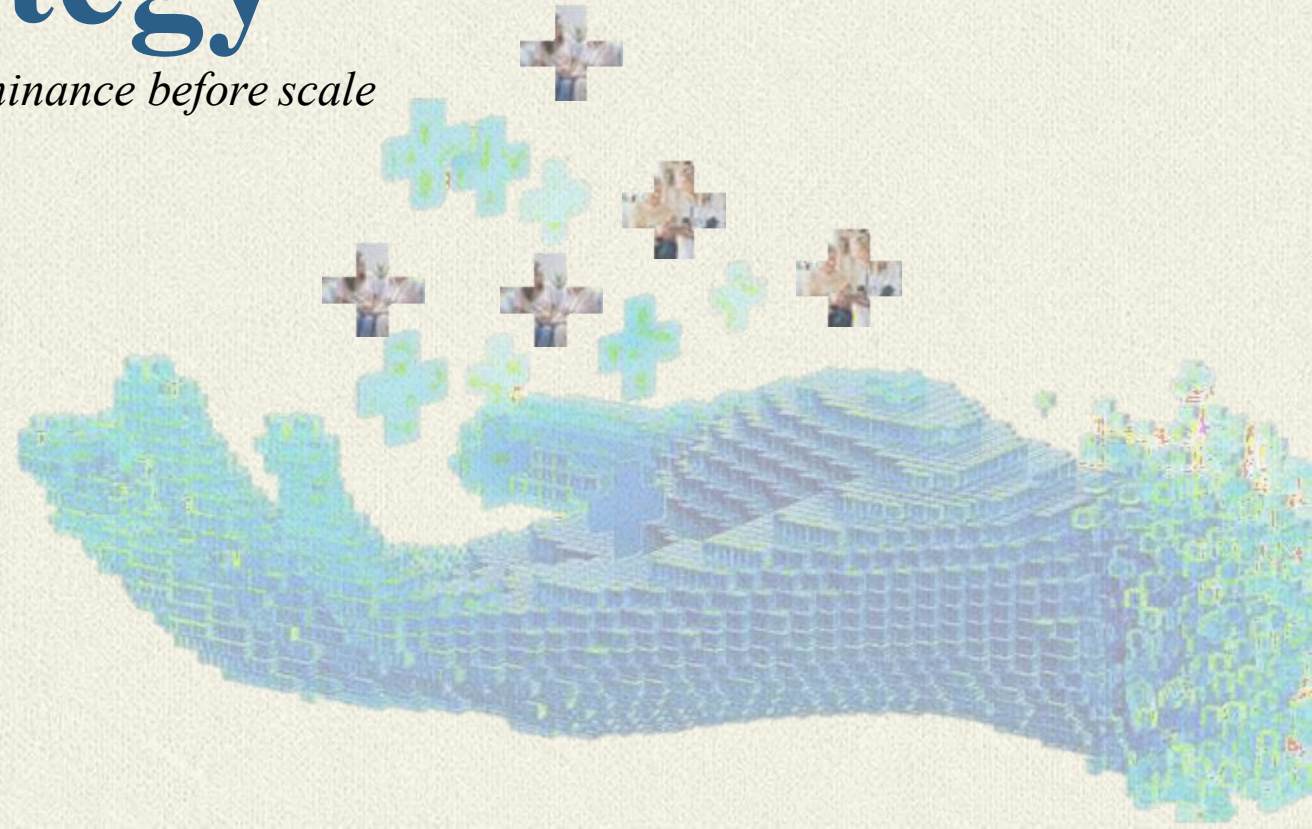
AI-search optimized

Near-zero CAC over time

Start with the wedge → build adoption → scale through channels

Dual GTM Strategy

Where we win first — beachhead market and niche dominance before scale



Dual GTM - Grow With AND Without Clinical

Growth does not depend on clinical activation

Track A

Intermediate Growth

Direct-to-consumer engine

Google Search (high-intent)

Caregiver communities (Facebook)

Retargeting + freemium funnel

- + Fast feedback
- + Scalable
- + No institutional dependency

Track B

Long Term Scale

Clinical trust engine

Physician referrals

Discharge touchpoints

Workflow integration

- + High trust
- + Lower CAC (long-term)
- + Slow to activate

Both engines run from Day 1 - growth is not gated by clinical

Competitive Defensibility

The honest version — what is and isn't a real moat



Defensibility - What Actually Holds

Not all moats are equal — some signal credibility, others create real defensibility

Moat	Reality	Strength	Our Response
Academic partnerships (MIT/Oxford)	Credibility signal — easy for big tech to replicate.	WEAK	Use in trust messaging only. Not a competitive defense.
Patent-pending AIP Score™	Short-term protection (~2–3 years) — can be worked around	MODERATE	File broadly, but don't rely on it
Family-centric architecture	Health tech ai is built for the adult child. hard to copy without rebuilding product	STRONG (2–4 yrs)	This is the core architectural moat. Design everything around it.
Longitudinal data asset	Becomes defensible only after sustained usage (12–18 months)	STRONG (12–18 mo+)	Focus on enrollment and retention early
Clinical workflow integration	First to integrate discharge workflow gains advantage	STRONG (3–5 yrs)	Move fast , Secure first partnerships
Brand trust — caregiver community	Builds slowly but compounds through word-of-mouth	STRONG (18 mo+)	Use wedge to accelerate trust

Defensibility is not built at launch - it is earned through usage, data, and trust over time

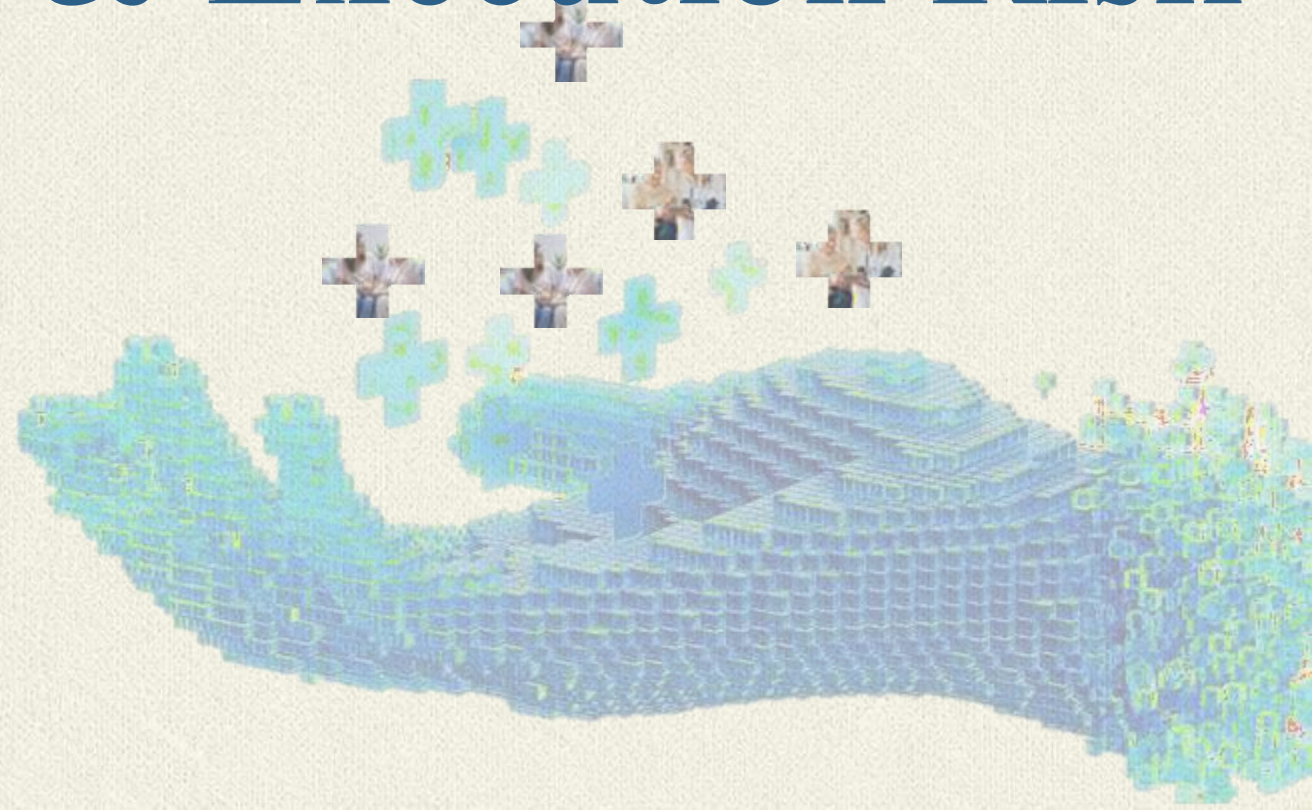
Weak

Moderate

Strong

Unit Economics & Execution Risk

Three scenarios · Named pivot triggers · Risk register



Unit Economics - Scenarios & Risk Triggers

Early CAC is \$65–120 — not \$35

The model works only if CAC stays below ~\$90

CAC Sensitivity by Scenario

Scenario	CAC	Conversion Rate	LTV:CAC	Payback Period
	\$35–50	15% completers	5–7x	2–3 mo
	\$65–90	10% completers	2.7–3.7x	4–5 mo
	\$120–160	5% completers	1.1–1.5x	8–10 mo

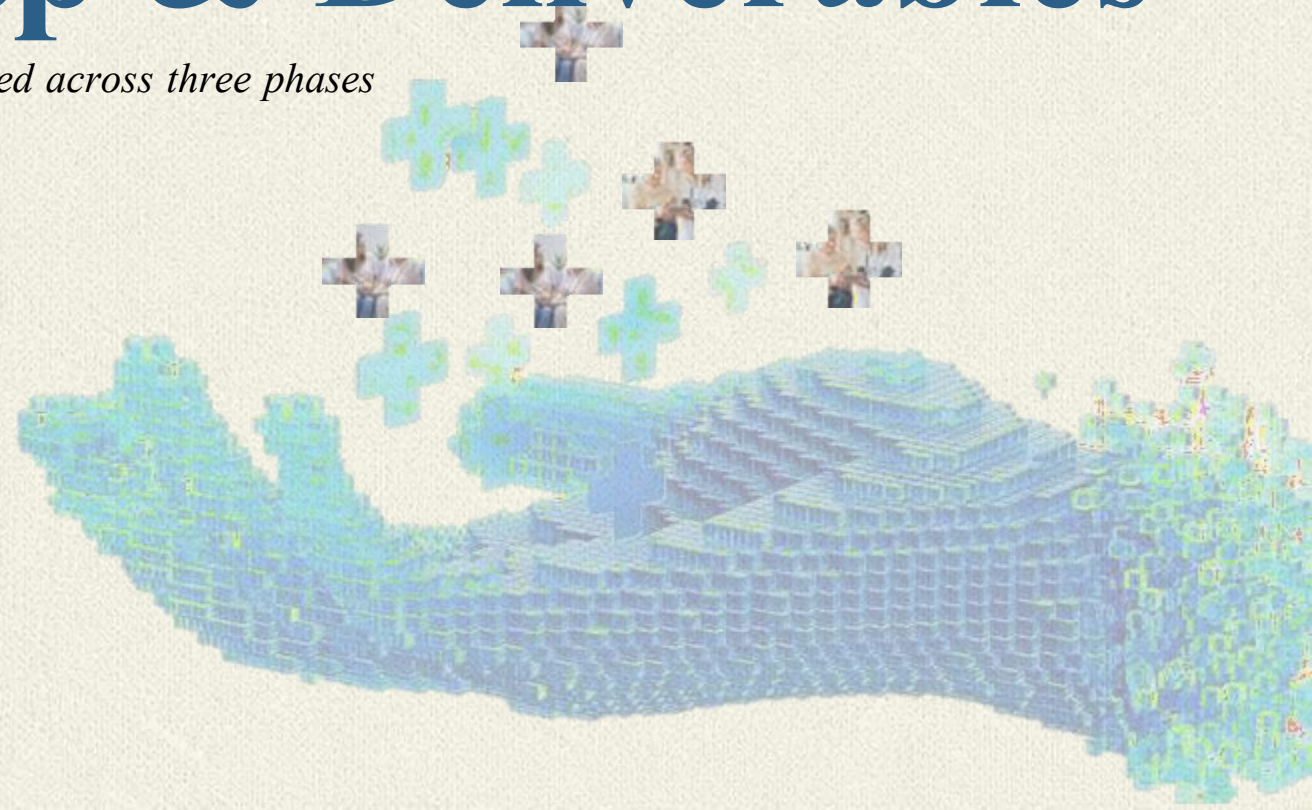
Execution Risk Register

	Completion <15% <i>Fix onboarding (Q3 friction + SMS)</i>
	CAC exceeds \$100 by Month 3 <i>Pause broad terms → focus wedge keywords</i>
	Day-14 conversion < 3% <i>Test pricing (\$9.99) + validate habit loop</i>
	Clinical pilot below digital baseline <i>Shift to credibility only (not acquisition)</i>
	Parent resistance <i>Launch caregiver-only mode</i>
	Regulatory/data privacy challenge <i>Pause + implement HIPAA compliance</i>

We don't assume success — we define when to pivot

90 Day Roadmap & Deliverables

Phase 1 = 80% of impact · Wedge-focused · All 8 gaps addressed across three phases



90-Day Plan - What We Do First

Wedge first. KPIs from Day 1. Clear pivot triggers.



Phase 1 — Days 1–30

80% OF YEAR-1 IMPACT

1

Brand relaunch: Emotional messaging, 'clinically-informed scoring, Story

2

AIP Score landing page live: Fix onboarding friction

3

Video testimonials : 3 Cognitive decline family testimonials

4

Google Ads: High intent Keywords

5

Clinical Pilot: Warm physician contacts - QR code only, no paperwork, organic signal

6

SMS Abandonment flow live



Phase 2 — Days 31–60

CLOSES THE GAPS

1

Habit Loop Live

2

Freemium Pricing test (\$9.99 / \$14.99 / \$19.99)

3

Family invite feature live: habit Loop 3 (network effect, churn -40–60%)

4

Facebook retargeting: NJ Alzheimer's caregiver communities + score-page abandoners

5

Clinical signal decision - go/no-go decision

6

CAC + conversion dashboard



Phase 3 — Days 61–90

COMPOUNDS + SCALES

1

4 SEO blog posts

2

YouTube Q&A: 2 episodes with Owner cognitive decline family focus

3

Clinical Month 3: if signal confirmed, begin ONE formal practice BAA negotiation

4

AI assistant optimization: structured data for ChatGPT/Claude/Copilot

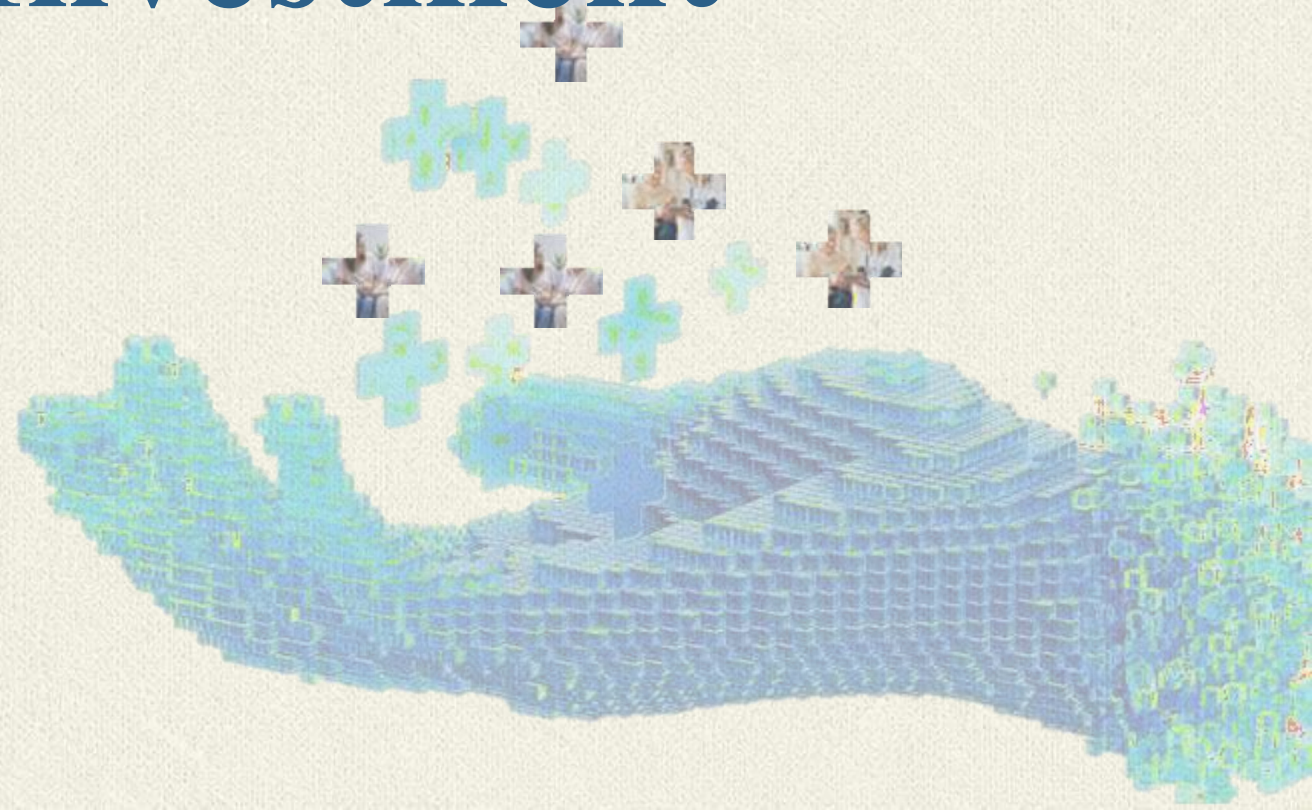
5

90-day KPI review: moat depth assessment, defensibility status

6

12-month growth roadmap: when to open high-income urban NJ/NYC

Media & Tools Investment



Media & Tools Investment (90-Day Plan)

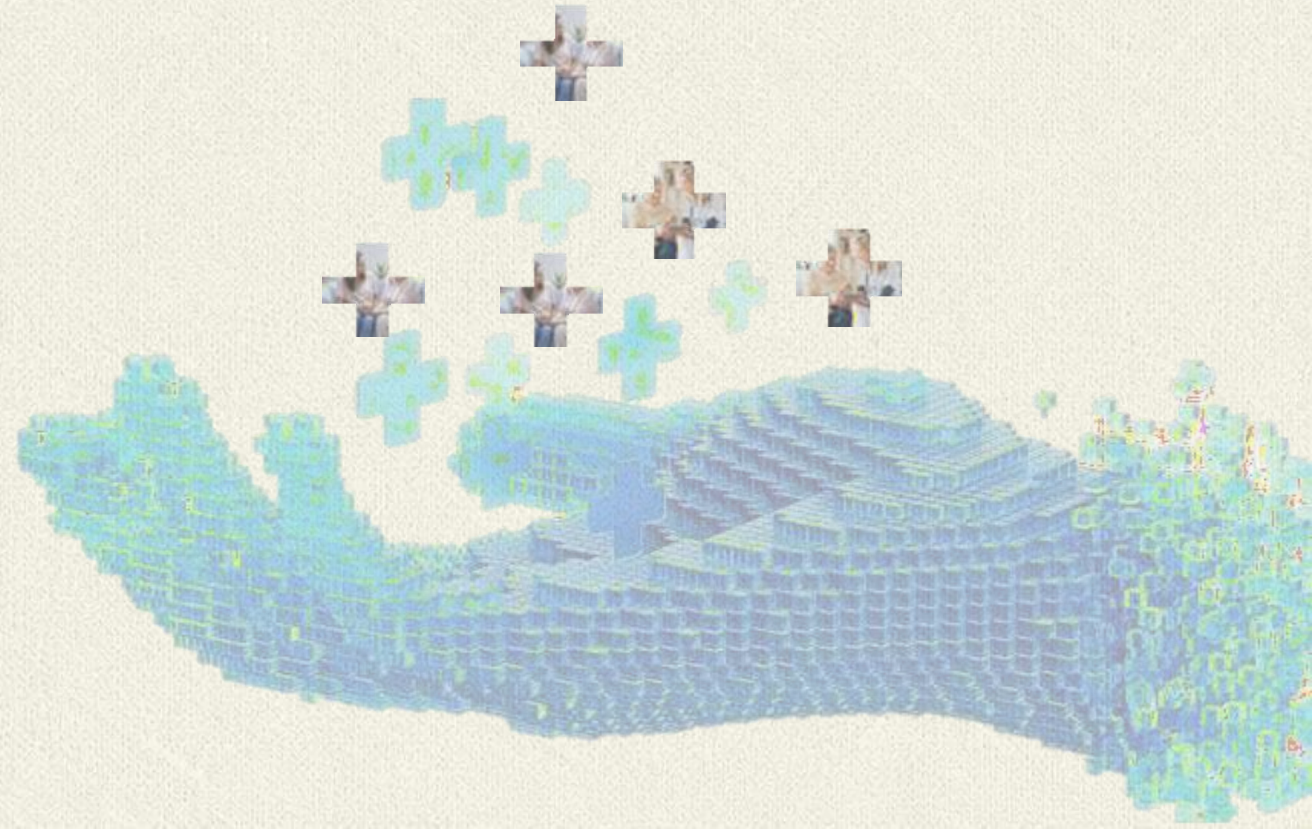
Budget Category	Month 1 (Days 1–30)	Months 2–3 (Days 31–90)	Notes
Paid Search — Google Ads (Wedge keywords)	\$1,500 – \$2,500	\$3,000 – \$5,000	High-intent terms only; no broad senior-care spend
Facebook / Meta Ads (Caregiver communities + retargeting)	\$800 – \$1,200	\$2,000 – \$3,500	NJ Alzheimer's groups, score-page abandoner retargeting
SMS Abandonment Tool (e.g., Klaviyo / Attentive)	\$150 – \$250	\$300 – \$500	2-hour re-engagement flow; ~\$150/mo base plan
Email Marketing Platform (e.g., Mailchimp / ActiveCampaign)	\$50 – \$100	\$100 – \$200	Habit loop emails, weekly score digest
CRM / Analytics Dashboard (e.g., HubSpot / Mixpanel)	\$100 – \$200	\$200 – \$400	CAC tracking, conversion funnel, 3-scenario bands
Video Production (3 caregiver testimonials)	\$2,000 – \$4,000	—	One-time; professional shoot, editing & captions
YouTube / Content Tools (e.g., Descript, Canva Pro)	\$50 – \$100	\$100 – \$200	2 episodes/mo; AI-search-optimised Q&A format
SEO / Content Tools (Ahrefs / Surfer SEO)	\$100 – \$200	\$200 – \$400	Wedge keyword research + 4 blog posts/month
Clinical QR & Print Materials (Discharge cards, physician one-pagers)	\$200 – \$400	\$100 – \$200	Low-cost; 2–3 physician warm contacts, no formal BAA yet
ESTIMATED TOTAL INVESTMENT	\$5,000 – \$9,000	\$6,000 – \$10,400	90-Day range: \$11,000 – \$19,400

Budget is intentionally focused on high-intent acquisition and validation — not broad awareness.

These are client-controlled media and tool costs. Ranges reflect the base-case scenario; worst-case (CAC > \$100) may require up to 30% increase in paid search spend. All figures are estimates for planning purposes.

Next Steps

From proposal signature to first deliverables in 14 days



Next Steps — From Alignment to Live Execution in 14 Days

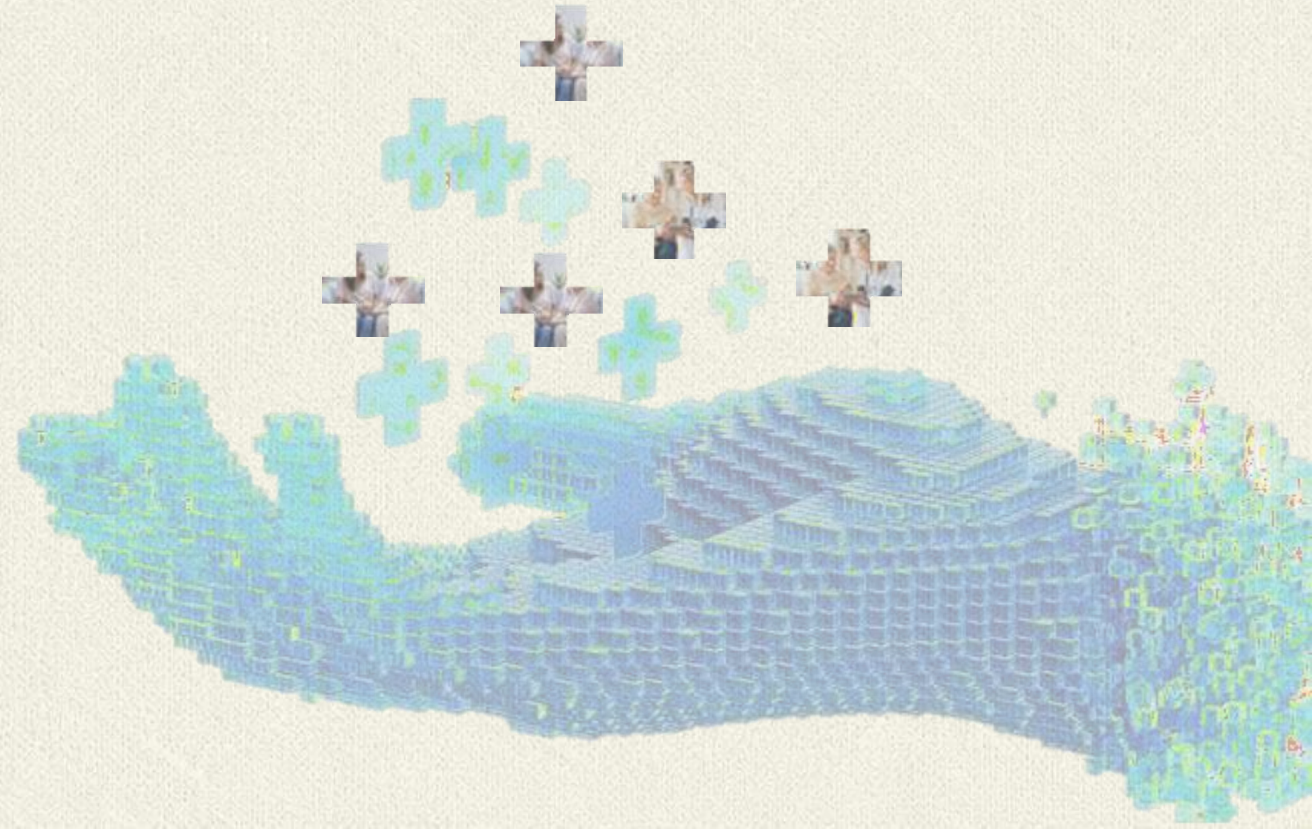
Clear actions · Clear owners · Immediate validation

Days 1–2	Days 3 - 4	Day 5 - 7	Day 8 - 14
Align & Confirm Direction	Setup & Baseline	Execution Kickoff	System Goes Live
Confirm Wedge focus (early cognitive decline families) Align on success metrics CAC target (\$65–90) Onboarding completion (18–25% → lift target) Day-14 conversion (~3%) Finalize 90-day execution scope	Agreement & milestone plan KPI dashboard baseline (3 CAC scenarios) Tracking setup (onboarding + paid + clinical pilot) Slack + working cadence	Lock messaging (guilt → identity → reassurance) Prioritize onboarding fixes (Q3 drop-off + SMS recovery) Finalize Google Ads wedge keyword clusters Confirm 2–3 warm physician contacts	Homepage relaunch (story-led + wedge messaging) AIP Score funnel live (completion fixes active) Google Ads live (high-intent wedge keywords only) Clinical QR pilot ready (warm contacts, no paperwork)

We start with validation, not scale — and adjust based on real signal

Appendix

From proposal signature to first deliverables in 10 days



Appendix A — Complete Survey Data (n=30, Qualtrics Export)

All 24 questions with precise figures from dashboard export

Questions 1–12

Q1: Relationship	Parent: 69% · Spouse: 3% · Sibling: 10% · Other: 17% (n=29)
Q2: Caregiving Role	Early stages: 39% · Primary: 29% · Co-caregiver: 25% · Remote: 7% (n=28)
Q3: Proximity	Within 30 min: 41% · Same home: 21% · 1-2hrs: 17% · 2+hrs: 21% (n=29)
Q4: Time in Role	<1 year: 37% · 1-3yrs: 30% · 3-5yrs: 15% · 5+yrs: 19% (n=27)
Q5: Trigger Moment	Memory/cognitive: 40% · Hospital: 27% · Family conv: 27% · Proactive: 27% · Fall: 23% (n=30)
Q7: Urgency	Somewhat urgent: 44% · Very urgent: 26% · Not urgent: 22% · Extremely: 7% (n=27)
Q9: Current Tools	Phone/video: 77% · In-person: 69% · In-house caregiver: 19% · Wearable: 19% · Aide: 15% (n=26)
Q10: Discovery Channels	Doctor/SW: 59% · Friends/family: 59% · Google: 44% · Other: 22% · Facebook: 11% (n=27)
Q12: AIP Usefulness	Extremely useful: 37% · Very useful: 30% · Moderately: 33% · Mean: 4.04/5 (n=27)
Q13: Best Message	'Stay one step ahead': 33% · 'Confident decisions': 33% · 'Truly safe': 30% · 'Last to know': 4% (n=27)

Questions 14–24

Q14: Tool Confidence	Reliable monitoring: 28% · Clear score: 20% · Other families: 20% · Doctors' input: 12% (n=25)
Q16: Brand Descriptor	Clinically-informed: 35% · Family intelligence: 31% · Data-driven peace: 23% · AI-powered: 8% (n=26)
Q17: Hesitation	Cost: 52% · Data privacy: 44% · Complexity: 33% · AI distrust: 33% · Parent resistance: 30% (n=27)
Q18: Trust Builders	#1 Family testimonials · #2 Clinical credentials · #3 Free trial · #4 Doctor endorsement (ranked, n=22)
Q19: Data Comfort	Comfortable: 39% · Neutral: 23% · Uncomfortable: 38% · Mean: 2.96/5 (n=26)
Q20: Price Ceiling	Under \$25: 33% · \$25–\$50: 22% · \$50–\$70: 22% · Free only: 15% · \$70+: 7% · Mean: 2.74/5 (n=27)
Q21: Free Score	Probably yes: 50% · Yes definitely: 35% · Unlikely: 15% (n=26)
Q22: Age	45–54: 38% · 55–65: 23% · 35–44: 19% · 65+: 19% (n=26)
Q23: Gender	Female: 78% · Male: 19% · Prefer not to say: 4% (n=27)
Q24: Income	\$150K+: 48% · \$50–99K: 32% · \$100–149K: 16% · Under \$50K: 4% (n=25)

Appendix B - References (ANA Format) | 19 Sources

AARP. (2024). Home and community preferences among adults 18 and older. <https://doi.org/10.26419/res.00831.001>

AARP. (2025). 2025 tech trends and adults 50+. <https://doi.org/10.26419/res.00891.001>

AARP. (2026). 2026 tech trends and adults 50+. <https://www.aarp.org/pri/topics/technology/internet-media-devices/2026-technology-trends-older-adults/>

Alzheimer's Association. (2024). 2024 Alzheimer's disease facts and figures. <https://www.alz.org/alzheimers-dementia/facts-figures>

Andreessen Horowitz. (2024). Health tech consumer benchmarks. <https://a16z.com/health-tech-benchmarks-2024/>

CNBC. (2025, November 21). The senior population is booming. <https://www.cnn.com/>

Cardinal Digital Marketing. (2025). In-home senior care PPC case study. <https://www.cardinaldigitalmarketing.com/>

Eyal, N. (2014). Hooked: How to build habit-forming products. Portfolio/Penguin.

Fogg, B. J. (2019). Tiny habits. Houghton Mifflin Harcourt.

HIMSS. (2024). Healthcare consumer digital engagement benchmark report. <https://www.himss.org/>

Hotjar. (2024). SaaS onboarding drop-off analysis. <https://www.hotjar.com/>

Kauttonen, J., Rousi, R., & Alamäki, A. (2025). Trust in AI healthcare. JMIR, 27, e65567. <https://doi.org/10.2196/65567>

Lieberman, M. (2024). How to write a consulting proposal. <https://www.melisalieberman.com/>

National Alliance for Caregiving & AARP. (2025). Caregiving in the U.S. 2025. <https://www.johnhartford.org/>

Population Reference Bureau. (2025). Seven trends reshaping aging in America. <https://www.prb.org/>

Rock Health. (2024). Digital health funding and market report. <https://rockhealth.com/>

Umekubo, P. (2026, March 27).XYZ investor deck



THANK YOU